

A Relational-Process Theory of Self-Stigma

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Abstract

Stigma is fundamentally a mechanism of social exclusion, yet dominant models of self-stigma locate its primary harm in psychological suffering. If stigma culminates in exclusion, self-stigma must concern the regulation of social participation, not only the disturbance of internal states. This paper proposes a Relational-Process Theory (RPT) of self-stigma, which reconceptualizes self-stigma as an adaptive process through which individuals regulate their own access to collective life under conditions of perceived exclusion. RPT is premised on three tenets: self-stigma is relational, regulatory, and defined by constrained participation. Stigma is integrated through evaluative processes (normative judgments of deservingness), prospective processes (the anticipatory constriction of perceived possibility), and embodied processes (the cumulative affective-physiological toll of inhabiting a stigmatizing world). Self-stigma emerges as patterned orientations toward engagement with the social world. RPT helps resolve longstanding tensions, including withdrawal without stereotype endorsement, embodied constraint without cognitive mediation, and self-stigma's persistence beyond stigmatizing labels.

Stigma is fundamentally a mechanism of social exclusion (Goffman 1963; Kurzban and Leary 2001). It marks individuals as undeserving of full personhood and the social goods that accompany it, organizing access to participation in collective life along lines of social acceptability. Although the form of the “mark” has evolved – from visible bodily inscriptions to more diffuse symbolic classifications – the social function of stigma has remained remarkably consistent: to signal non-belonging and regulate the boundaries of membership.

In its subtler manifestations, stigma appears as glances of disapproval, strained interactions, or one’s implicit sense that they do not fully belong (Ilic et al. 2013; Young et al. 2019). In its more overt forms, stigma materializes through discriminatory policies, interpersonal rejection, and institutional barriers. Across these variations, however, the underlying logic is the same. The logic, the outcome, and the core harm of stigma is exclusion from full participation in social life.

Contemporary scholarship increasingly recognizes stigma as a social process rather than merely a collection of negative attitudes. Most notably, Link and Phelan reframed stigma as a dynamic sequence culminating in status loss and discrimination, shifting analytic attention from prejudice alone to the mechanisms through which social hierarchies are reproduced (Link and Phelan 2001). This perspective clarified an essential point: stigma is not simply something that is believed – it is something that happens.

Yet while the social nature of stigma has achieved considerable conceptual clarity, the concept of *self-stigma* has remained comparatively unsettled. Prevailing definitions typically describe self-stigma as stigma that has been internalized – a psychological state in which individuals adopt the stereotypes, prejudices, and devaluations they perceive society to hold about them (Corrigan and Rao 2012; Lucksted and Drapalski 2015; Nutter, Saunders, and Waugh 2024). Consequently, self-stigma is most often operationalized as a constellation of attitudes, beliefs, and affective responses reflecting agreement with negative social evaluations (Boyd Ritsher, Otilingam, and Grajales 2003).

Some theoretical models have advanced this formulation by conceptualizing self-stigma as a process rather than a static psychological phenomenon (Corrigan et al. 2012; Corrigan and Rao 2012; Luoma et al. 2013). These models importantly expand the construct beyond belief endorsement to include intrapersonal experiences like identity transformation and experiential avoidance. Psychological distress is thus positioned as both a defining feature and a principal consequence of self-stigma.

These developments represent meaningful progress. However, they remain incomplete.

Across otherwise divergent frameworks, a shared assumption persists: that the core harm of self-stigma resides in psychological suffering.¹ This assumption introduces a quiet but consequential contradiction. If stigma, as a social process, leads to exclusion, then self-stigma must be understood as the individualized instantiation of that exclusion. Psychological distress is consequential, but it is not constitutive of self-stigma.

Rather, by definition, self-stigma must concern the regulation of social participation.

This paper argues that self-stigma should be reconceptualized accordingly – not as a property of individuals, nor as a collection of internal states, but as an adaptive process through which persons regulate their own access to social life under conditions of perceived exclusion. Self-stigma does not primarily describe how individuals feel about themselves; rather, it reflects how they orient toward, approach, or withdraw from participation in the social world.

Relocating the core harm of self-stigma from distress to constrained participation resolves several longstanding ambiguities within the literature. It clarifies what counts as self-stigma and what does not, distinguishes the process from its emotional correlates, and explains why changes in belief or affect do not necessarily restore engagement. Most importantly, it restores conceptual continuity between stigma as a societal mechanism of exclusion and self-stigma as its lived enactment.

The Relational-Process Theory of Self-Stigma is premised on three ontological tenets. First, self-stigma is relational: it concerns how individuals understand their position within a social environment structured by stigma.² Second, self-stigma is regulatory: it is enacted through ongoing evaluations of legitimacy, expectations of acceptance or rejection, and embodied assessments of one's capacity to participate. Third, the defining harm of self-stigma is the constraint of participation in collective life.

Together, these tenets circumscribe what "counts" as self-stigma and address the definitional ambiguity that has long characterized the construct. Self-stigma is neither merely internal nor purely social; it is the adaptive regulation of participation among those who come to experience themselves as vulnerable to exclusion.

The theory and conceptual model that follow elaborate this ontology by asking how socially encountered stigma becomes organized within the self. Stigma is encountered through direct experience, observation, and the broader symbolic environment. These encounters shape how people perceive their place in the social world: whether they feel entitled to participate, whether participation seems likely to be welcomed, and whether engagement feels tolerable or possible. Through this process, stigma is not simply absorbed – it is enacted, as individuals calibrate their engagement with the social world. Self-stigma, in this view, is literally what one does to oneself in response to stigma. It is the gradual fulfillment of society's expectation of social exile.

Intellectual foundations

The Relational-Process Theory (RPT) of self-stigma does not emerge in a vacuum; rather, it synthesizes and extends several influential traditions in stigma scholarship to account for the lived enactment of self-exclusion.

Structural and social grounding

Self-stigma is not an internal phenomenon isolated from its environment; it emerges in social context. RPT draws its structural grounding from the work of Link and Phelan (2001), who reframed stigma as a social process eventuating in separation and status loss. This shift from individual prejudice to social mechanism

clarifies that stigma is not merely a belief but something that *happens* in the world, and this *happening* produces and sustains inequality (Earnshaw et al. 2022; Hatzenbuehler, Phelan, and Link 2013; Pescosolido and Martin 2015). Parker and Aggleton (2003) further inform this orientation by emphasizing that stigma is fundamentally embedded in systems of power and social hierarchy, providing the necessary context for RPT's focus on legitimacy, positioning, and constrained participation. Finally, Yang's "what matters most" framework positions stigma as a cultural and moral statement of belongingness that gatekeeps access to values expression in local social worlds (Sibley et al. 2025; Yang et al. 2007, 2014).

The shift toward process

The study of stigma mechanisms, the various ways stigma 'gets under the skin,' was advanced by Earnshaw and Chaudoir (2009) and Hatzenbuehler (2009), among others. These models elucidated that the health and social impacts of stigma, as experienced by the stigmatized, are multi-sourced. At the individual level, Corrigan and colleagues' progressive model of self-stigma has been seminal in understanding the process of identity-based internalization, providing an important roadmap for how individuals move from stereotype awareness to self-concurrence and diminished goal pursuit (Corrigan, Larson, and Rüsch 2009; Corrigan, Rafacz, and Rüsch 2011). Luoma and colleagues (2013) advanced this work by explicitly conceptualizing self-stigma as a dynamic psychological process of internalization and anticipation that hinders the pursuit of life goals. RPT builds on these insights while proposing a broader regulatory architecture. Whereas existing models have often placed internalized belief as the primary mechanism of self-relevance, RPT integrates evidence from the minority stress and stereotype threat traditions, which demonstrate that stigma exposure can produce sustained vigilance, cognitive load, and physiological strain even in the absence of explicit stereotype endorsement. Pachankis's model of stigma concealment (2007) captured the cognitive-affective-behavioral dynamic of hiding a concealable identity, an act that requires vigilance and impression management. Self-stigma, too, should be understood as an active and activating process.

Pre-reflective regulation

Finally, the theory is conceptually informed by Bourdieu's (1990) account of habitus and practical sense. This perspective underscores how social hierarchies become sedimented into pre-reflective orientations toward action—what individuals come to take for granted as their place in the social world. Goldberg (2017), in a Bourdieusian turn, notes that stigma is at once pervasive and invisible: It is pernicious in effect yet broadly unnoticed as exceptional. Major and O'Brien's model of identity threat (2005) further emphasizes that stigma appraisals need not be volitional nor accurate to be harmful. This sensibility supports RPT's emphasis on the tacit, embodied calibration of participation rather than a purely deliberative or cognitive decision process.

Taken together, these foundations support a model that connects structural exclusion to the individual enactment of adaptive self-exclusion. Guided by these traditions, the Relational-Process Theory finds its three ontological tenets of self-stigma as relational, regulatory, and an intrinsically social act.

The dynamics of self-stigma: Regulatory processes and social participation

Self-stigma is reconceptualized in RPT as a dynamic and adaptive regulatory repertoire that governs an individual's engagement with the social world. Regulation, in this context, refers to the continuous calibration of one's readiness and perceived permission to extend oneself into collective life – a concept that encompasses the full range of activities through which individuals occupy social roles and enact core values, from earning a living and moving through public spaces to forming relationships, practicing faith, and expressing oneself openly. This regulatory process is organized around three primary parameters, which may take the form of deliberate judgments or more automatic, felt orientations. *Legitimacy* – the evaluative parameter – is a normative assessment of one's social deservingness or belongingness (*Should I?*); it renders participation either as warranted or as a violation of social norms. *Expectancy* – the prospective parameter – is a probabilistic assessment of the likelihood and consequences of rejection (*Will I be permitted to?*); it often functions as learned pattern recognition that constrains imagined possibilities before they can materialize. *Capacity* – the embodied parameter – is an affective-physiological assessment of one's readiness and energetic resources (*Can I?*); it accounts for how the accumulated somatic toll of stigma can make engagement feel too burdensome to

pursue, regardless of belief or expectation. Together, these appraisals constitute the performance of self-exclusion, shaping whether an individual fully participates in, selectively navigates, or withdraws from collective life (Figure 1).

[[[FIGURE 1 ABOUT HERE]]]

Stigma cues

Implicit in the model is the assumption that social stigma is felt or noticed; before stigma can be evaluated, anticipated, or embodied, it must first be registered. Stigma cues are environmental signals that inform how individuals understand their social position in relation to a stigmatizing mark. They function as explicit or implicit messages that exclusion is possible, likely, or warranted. The concept of the *mark* – the attribute targeted for stigma – is analytically distinguished from group identity. While group identification may amplify or organize self-relevance, the present framework centers the perception that one is liable to stigma on the basis of a discreditable characteristic or behavior. Identity processes are therefore understood as one possible pathway through which stigma becomes self-relevant, rather than a necessary precondition.

Experienced stigma is typically the most visceral and immediate form of cue exposure. Overt acts of interpersonal prejudice or discrimination (e.g., from family members, employers, or healthcare providers) are directly registered and integrated. Stigma may also be encountered *vicariously* through observed instances of interpersonal devaluation, whether witnessed in person or via mediated contexts. Vicarious stigma thus operates through social learning about the consequences of being marked.

Finally, stigma may be registered in the absence of discrete stigmatizing events. *Symbolic* stigma includes diffuse, culturally embedded signals that communicate who is socially devalued and where belonging is conditional. These signals are conveyed through patterned media representations, institutional narratives, public discourse, spatial arrangements, and taken-for-granted social norms.

Taken together, stigma cues constitute the perceptual³ inputs through which social stigma becomes registered and integrated into self-stigma processes.

Evaluative self-stigma

Evaluative self-stigma refers to the integration of stigma cues into one's sense of social deservingness or belongingness.⁴ It operates through a legitimacy appraisal – a normative assessment of whether one's presence in collective life is warranted. Within the regulatory architecture of self-stigma, evaluative processes constrain participation by rendering social exclusion intelligible, appropriate, or even justified. What matters is not the origin of the belief but its relational consequence: the perceived illegitimacy of one's claim to social participation.

The endorsement of stigma legitimacy may arise through multiple pathways. Individuals may come to accept judgments about their social standing with or without adopting a stigmatized identity. While identity processes can serve as a conduit through which perceived social beliefs become self-relevant, they are not theoretically central to the present framework. Evaluative self-stigma concerns the normative organization of participation rather than the psychological consolidation of identity. So long as the individual perceives judgments of non-belongingness to reflect a social truth, participation may be adaptively constrained.

Negative affect is not constitutive of evaluative self-stigma. Normative assessment does not require self-devaluation. Role-making and role-taking are defining features of organized societies and may be accepted and taken for granted as natural. Although shame and diminished self-esteem frequently co-occur with such judgments – and may function as mechanisms through which stigma becomes embodied – these affective states are not intrinsic to the relational process itself. Social legitimacy judgments should therefore be analytically distinguished from affective self-assessment.

Evaluative self-stigma is thus analogous to, though conceptually distinct from, internalized stigma. Whereas internalized stigma is often defined as the adoption of negative beliefs about the self, evaluative self-stigma centers the regulatory consequences of accepting non-deservingness as legitimate.⁵

Prospective self-stigma

Prospective self-stigma structures the field of perceived possibility. It is a future-oriented constriction of social participation based on the presumption of rejection, denial, or exclusion. Past discrimination, vicarious lessons of rejection, and tacit cultural signals about who is welcome function as perceptual inputs for expectations of inclusion. Here, stigma cues come to register as evidence shaping participation intentions and endeavors. The implicit question guiding prospective self-stigma is: “What will happen if I try?” Non-participation reflects the felt likelihood of rejection, what is perceived to be at risk in attempting participation, and the anticipated consequences of exclusion. Thus, prospective self-stigma operates through the regulatory parameter of expectancy. Expectancy need not take the form of explicit reasoning; it is better understood as learned pattern recognition. It emerges as a practical orientation – often pre-reflective, affectively charged, and sedimented through experience – that governs what feels possible. Expectancies may be situational (“my doctor”), categorical (“doctors”), or general (“all people”) in reference. The prospect of exclusion may be probabilistically accurate, overgeneralized from limited experience, or maintained even in the presence of disconfirming evidence. The ostensible accuracy of prediction is non-essential; what matters is that the process calibrates willingness to extend oneself into social participation, often favoring protective withdrawal when exclusion is anticipated. In sum, prospective self-stigma reorganizes the social environment into one which is experienced as unnavigable, limiting imagined possibilities before they can materialize.

Embodied self-stigma

Embodied self-stigma refers to the accumulated affective-physiological toll of inhabiting a stigmatizing social world, through which participation becomes experienced as burdensome. As stigma cues accrue over time, the

body adapts to anticipated threat, and exclusion becomes registered not only cognitively but somatically – as tension, vigilance, depletion, or emotional numbing. Social engagement may therefore require sustained threat monitoring, gradually increasing allostatic load and narrowing regulatory flexibility.

The regulatory parameter of embodied self-stigma is capacity: one's perceived and felt readiness to extend oneself into social participation. Thus, individuals may regard themselves as deserving of opportunity or perceive participation as available, yet still withdraw when engagement is felt to exceed their physiological resources. Here, non-participation reflects not a judgment of illegitimacy nor an expectation of rejection, but an adaptive conservation of energy in the face of anticipated strain. If the evaluative and prospective elements of self-stigma constitute the mind's integration of stigma, the embodied component represents stigma as lived in the body. The guiding question becomes: "Can I tolerate participation?" or experientially, "Do I have it in me?"

Non-participation: The relational enactment of self-stigma

Relational enactment captures the behavioral expression of self-stigma through adaptive self-exclusion. Patterns of social (non)participation should not be understood as one of the many outcomes of self-stigma but as its central and defining manifestation. If the perception of stigma cues is the relational gateway through which one's inclusion in a stigmatizing environment is registered, then (non)participation is how the individual relates back to that environment. This adaptive style of accommodation fulfills society's implicit expectations. Self-stigma can thus be framed as the performance of self-exclusion produced through an orientation toward social participation that is shaped by evaluative, prospective, and embodied integration of social stigma cues.

The relational enactment of self-stigma can take multiple forms. At the most immediate level, individuals may engage in relational withdrawal, reducing their presence and visibility within stigmatizing environments – pulling away from trusted others, avoiding spaces of potential contact, and keeping to oneself. At a second level, individuals may practice agentic inhibition, silencing their own initiative by foregoing the pursuit of opportunities in important social domains such as education, employment, health care, and social network

formation. Finally, individuals may experience aspirational contraction, revising what they perceive as possible or appropriate to desire. This represents the motivational counterpart to values disengagement: the gradual recalibration of wanting itself. Across these forms, the guiding question of relational enactment becomes: “What do I allow myself to do?”

The relational enactment of self-stigma is a patterned orientation toward participation (Livengood and Stodolska 2004; Wolfe et al. 2017). Individuals may fully withdraw from stigmatizing environments, reducing their presence and visibility in social life. Others may selectively participate, rerouting valued activities into lower-risk or less publicly legible contexts where stigma exposure is reduced (e.g., online education, peer social spaces, telehealth, interest-based communities). Still others may pursue strategic or negotiated participation, deliberately managing disclosure, visibility, or setting to preserve conditional access to opportunity. At the most expansive end, some will continue to pursue full participation despite the strain of self-stigma. These patterns are not merely outcomes but observable expressions of how evaluative, prospective, and embodied processes regulate one’s extension into the social world. What appears as diminished motivation or aspiration may therefore reflect adaptive reorganization of participation under conditions of stigma.

On goals and values

Explicit in most models of self-stigma is the expectation that stigmatized individuals will gradually disengage from their values and abandon the pursuit of important life goals (Corrigan et al. 2009; Luoma et al. 2013). From the perspective of stereotype endorsement and internalization, goals and values are internal possessions that can be damaged by stigma. An individual *had* motivation, then stigma *took it away*. Interventions are designed to restore the individual’s belief in themselves – to repair the ‘damage’ within.

Yet goals and values are better understood as processes rather than states. They are constantly practiced, revised, and reinforced. Most importantly, they are enacted socially. Empathy, loyalty, tolerance, love, family, leadership, professionalism, accountability are values most fully realized and stabilized through social

expression. When access to full participation is constrained, individuals lose not only opportunities but also the relational context in which competence, agency, and aspiration are exercised and recognized. Over time, repeated non-enactment may produce an attenuated attachment to previously held values – not because motivation was internally extinguished, but because the social conditions necessary for its expression have been systematically restricted. What appears as diminished drive or internalized inadequacy may therefore, in some cases, reflect the cumulative effects of participatory attrition rather than self-devaluation.

Importantly, stigma may constrain the domains in which values are enacted without extinguishing the values themselves. Some stigmatized individuals may continue to express core values in private or protected spaces even as they disengage from more publicly evaluative domains. Individuals who report hobbies as a primary stigma coping mechanism (Sibley et al. 2025) may not simply be engaging in distraction but practicing valued identities in spaces where they are less liable for judgment.

Consider an example: A person who uses drugs previously worked in food services as a cook. They genuinely value culinary skill and creativity. As stigma constrains their access to employment – failed background checks, visible signs of use, being known in a small community – they stop pursuing restaurant work. Under many existing models, the “why try” effect would be the primary available explanation: they have internalized that “people like me can’t hold a job” (Corrigan et al. 2009). Yet at home, in an encampment, or at a harm reduction drop-in, they continue cooking elaborate meals, experimenting with recipes, and taking pride in feeding others. The value remains intact. What has been lost is access to the social field in which that value is practiced, recognized, and institutionally rewarded.

Whether values disengagement manifests through internalization or diminished opportunities for social enactment, the distinction is not merely semantic. A clinician, case manager, or researcher using existing self-stigma instruments may mistakenly assess these individuals as having low vocational motivation or diminished life goals, missing the ongoing enactment in a domain that isn't legible to institutional observers.

And if values aren't damaged internal states but under-enacted processes, then the intervention target shifts from cognitive repair to creating participatory openings. Peer employment, inclusive social spaces, and mutual aid networks do more than promote empowerment in the conventional sense; they provide alternative social infrastructures where values can be practiced and recognized.

In this way, RPT reinterprets what has long been treated as one of self-stigma's most definitive outcomes – the erosion of goals and values – as, at least in part, a consequence of structural exclusion from the conditions under which goals and values are sustained.

Unresolved tensions in contemporary self-stigma scholarship

Despite decades of conceptual refinement, the field remains theoretically unsettled. While there is broad agreement that self-stigma concerns how individuals respond to social stigma, there is far less consensus regarding what self-stigma fundamentally is. Across the literature, it is alternately treated as a belief, an affective state, a trait, a process, a behavioral outcome, or some composite thereof. Many influential models implicitly traverse these levels of analysis, often collapsing them into multidimensional scales without fully specifying their ontological status. This ambiguity is not merely semantic. It generates recurring conceptual tensions that existing frameworks only partially resolve.

Evaluation without endorsement

Those marked for stigma may evaluate themselves as unworthy of full participation without internalizing negative self-beliefs.

Self-stigma is commonly defined as the internalization of negative stereotypes. Yet individuals frequently disengage from social life without endorsing those stereotypes as true. They may explicitly reject stigma as unjust while nonetheless refraining from applying for jobs, pursuing relationships, or seeking services. If

stereotype agreement is the defining feature of self-stigma, such cases are theoretically anomalous. Belief endorsement cannot be the sole mechanism of self-exclusion.

Through evaluative self-stigma, individuals may arrive at legitimacy judgments about their participation without global stereotype endorsement or identity adoption. What constrains participation is not self-hatred per se but the normative appraisal of deservingness. That is, the stigmatized may come to believe that their presence violates a social norm.

So long as legitimacy is centered as the regulatory parameter governing participation, evaluative self-stigma may manifest in multiple ways. The foremost is the progressive model proposed by Corrigan and colleagues (2011) in which, contingent on stereotype awareness, one agrees with and applies those stereotypes to oneself. Other pathways are imaginable, however, if we shift the fundamental question of self-evaluation from “What am I?” to “What is my place in this world?” Then the topic of evaluation is not the agreeance of specific stereotype content (e.g., competence, dangerousness, blame) but what the imposition of this content suggests about a person’s social standing. This is internalization of hierarchy rather than internalization of belief. If legitimacy of exclusion can be accepted without stereotype endorsement, then there are ostensibly two additional mechanisms of non-participation: one believes that hierarchical demotion reflects an objective reality (non-participation as social truth), or one comes to accept that their presence is a burden or imposition on others (non-participation as social etiquette). Self-devaluation becomes a common, though not essential, component of normative self-regulation.

Empirical work underscores this distinction. In a principal component analysis of the Internalized Stigma of Mental Illness scale (Boyd Ritsher et al. 2003) among individuals with psychotic disorders, ‘experiential’ stigma (comprising alienation, social withdrawal, and discrimination experience) emerged as a distinct component that was significantly associated with affective symptoms and vocational status, whereas stereotype endorsement was not significantly correlated with these functional indicators (Schwarzbald et al. 2021). Although these findings do not adjudicate causal ordering, they are consistent with the possibility that

participation-relevant constraint may be more tightly linked to relational dimensions of stigma than to belief endorsement alone.

Expectation without legitimacy

Current self-stigma models do not account for withdrawal absent internalization.

Individuals often anticipate rejection without regarding exclusion as deserved (Corrigan and Watson 2002; Link et al. 1989). They may believe stigma is wrong, illegitimate, or immoral and still expect to be turned away. Anticipatory withdrawal therefore does not require evaluative agreement. Prospective regulation can operate independently of normative endorsement. Models that treat anticipated stigma as secondary to internalization miss this distinction.

Under the present definition of self-stigma (a process through which social non-participation eventuates), prospective stigma is not a separate phenomenon but one of the core processes through which self-stigma is integrated and enacted. The paradox of *expectation without legitimacy* was foreshadowed in Modified Labeling Theory (Link et al. 1989). What has been missing is the integration of socialized expectations as a self-stigma pathway distinct from internalization. If evaluative regulation has to do with endorsement of stereotypes, prospective regulation involves endorsement of meta-stereotypes (Klein and Azzi 2001): the recognition that stigmatized beliefs exist in the social world, that one is liable to be labeled with these beliefs, and that such labeling is grounds for exclusionary treatment. This, for some, may be enough to preclude full participation.

Attempts have been made to relate internalized and anticipated stigma as sequential steps toward poor health. Such studies disagree on whether anticipation leads to internalization (Fox, Smith, and Vogt 2018; Hing and Russell 2017; Quinn, Williams, and Weisz 2015) or vice versa (Benz et al. 2021; Lo Hog Tian et al. 2023; Turan et al. 2017). Outcomes are further obfuscated by the unreliability of attitudinal and affective serial mediation models: cross-sectional models are intrinsically biased (Maxwell and Cole 2007), while even longitudinal models struggle to capture the recursive, time-sensitive interactions among internal states

(Borsboom 2017; Fiedler, Schott, and Meiser 2011; Yuan et al. 2023). It is preferable to consider internalizing and anticipatory processes as synergistic rather than sequential, an arrangement that has some empirical support (Jahnke, McPhail, and Antfolk 2024; Moore 2016; O'Donnell and Foran 2024). Drawing on the sociological lineage of Mead and Blumer, Link and colleagues (2015) proposed 'symbolic interaction stigma' to account for anticipation of rejection as distinct from internalization. Their early empirical research provided strong support for this distinction yet has since remained underdeveloped. The present theory's repositioning of self-stigma aims to revive attention to these phenomena.

Constraint without cognition

Individuals may withdraw from social participation due to affective-physiological burden associated with stigma exposure even when they do not endorse stigmatizing beliefs about themselves and do not explicitly expect rejection.

A third unresolved tension concerns whether withdrawal necessarily depends on the endorsement of negative self-beliefs. Prevailing models often assume that diminished participation follows from internalized stereotypes or anticipated rejection. Yet converging evidence suggests that constraint may emerge independently of cognitive pathways (Dong et al. 2023; Loecher et al. 2025).

A growing body of research demonstrates that exposure to stigma and discrimination is associated with dysregulation across multiple physiological systems (Busse et al. 2017; Mijas et al. 2021; Porges 2021; Richman and Lattanner 2014; Wu and Berry 2018). Evidence from the everyday discrimination literature suggests that even subtle, chronic exposures to stigmatizing treatment are associated with cumulative physiological wear, including dysregulated cortisol patterns, hypertension, and sleep disturbance (Graham 2024; Lawrence et al. 2022; Van Dyke et al. 2020).

Research on stereotype threat demonstrates that merely being reminded of a negatively stereotyped identity can heighten vigilance, emotional regulation demands, and cognitive load, impairing performance and engagement

even among individuals who do not personally endorse the stereotype (von Hippel et al. 2017; Steele, Spencer, and Aronson 2002; Woodcock et al. 2012). The mechanism is situational and embodied: exposure to stereotype-relevant cues induces stress responses and attentional shifts that alter behavior independent of explicit belief change. Indeed, individuals need not subscribe to the stereotype for it to affect functioning (Woodcock et al. 2012).

Related work on everyday discrimination, minority stress, and John Henryism similarly documents physiological wear associated with chronic exposure to stigmatizing environments (Flentje et al. 2020; Hatzenbuehler 2009; Merritt, McCallum, and Fritsch 2011). These findings indicate that stigma can accumulate in affective-physiological systems through repeated vigilance, ambiguity, and role strain, gradually constraining behavioral flexibility. Importantly, many such studies do not establish that stereotype endorsement is the operative mediator, leaving open the possibility that participation becomes costly through embodied pathways alone.

Taken together, this evidence suggests a constraint-without-cognition pathway: social engagement may contract not only because individuals view exclusion as legitimate or expect rejection, but also because participation becomes experienced as energetically or physiologically burdensome, an experience that becomes chronic with time (Dyar 2024). The present framework does not claim that cognition is absent in these processes; rather, it highlights that existing models insufficiently account for how stigma becomes regulatory through the body itself.

The endurance of self-stigma

In an enlightening study of participants living with dual diagnoses of mental illness and substance use disorders, Link and colleagues found that stigma persists well beyond the formal shedding of the mark or label (Link et al. 1997; Patel et al. 2024). Their prescient reflection at the study's conclusion was: "Does the effect endure because rejection by others continues unabated? Is it difficult for the labeled person to shake off the mark and

the personal meanings attached to it? Or is it the trauma and pain of past rejection that stays with the stigmatized person?" (Link et al. 1997) The present theory suggests an answer that Link surmised thirty years ago: Likely, all of the above. By reframing self-stigma as the inertial regulation of social participation, the persistence of stigma's effects becomes theoretically expected rather than anomalous. A regulatory system does not unlearn its parameters overnight. For stigmatized diagnosable conditions that suppose a state change (an incomplete heuristic of intrapersonal evolution), there is not a magical return to inclusion. The stigmatized, whatever their labeled status, has long since accumulated evidence of non-belongingness, cultivated anticipatory schema, and inhabited a body violated by society's condemnations. What persists is not merely the memory of stigma but the sedimented orientations toward social engagement that stigma has helped to organize.

Boundary ambiguity, construct contamination, and the issue of aggregation

The present framework is intentionally circumscribed. Self-stigma is deeply intertwined with a range of emotional, cognitive, and behavioral phenomena, yet not all such phenomena are constitutive of self-stigma. Clarifying these boundaries is essential for theoretical coherence and for the development of valid measures and interventions.

Shame, though frequently associated with stigma, is not synonymous with self-stigma. Self-stigma without shame may manifest as resignation in the evaluative domain or fatalism in the prospective domain. Even within embodied processes, shame is neither necessary nor sufficient: the accumulated somatic toll of repeated exposure to stigma may alone constrain participation. As an internally directed affective state, shame does not inherently govern action. It may intensify or mediate self-stigma processes, but it does not itself perform the work of exclusion.

Moreover, shame may be decoupled from stigma altogether. One may feel profound shame over personal failures, interpersonal disappointments, or unmet goals without interpreting these experiences through the lens

of social exclusion. In such cases, the emotion is not relational in the sense required by RPT. Self-esteem similarly fails as a reliable marker of self-stigma. An individual may retain a strong global sense of self-worth while nevertheless experiencing fatalism regarding participation or feeling physiologically incapable of engagement. Conversely, diminished self-esteem does not necessarily reflect the adaptive regulation of social participation in response to stigma cues.

Shame and self-esteem are therefore best understood as potential mediators or correlates of self-stigma, not as components of the relational process itself. The present theory leaves open the possibility of testing these pathways empirically.

Nonetheless, across widely used measures, self-stigma is operationalized as a composite of heterogeneous elements – including the aforementioned affective responses (e.g., shame and self-esteem decrement), beliefs (e.g., stereotype endorsement), cognitive responses (e.g., experiential avoidance), experiences (e.g., discrimination), and behavioral tendencies (e.g., non-disclosure and withdrawal) (Boyd Ritsher et al. 2003; Corrigan et al. 2012; Lillis et al. 2010; Luoma et al. 2013; Yang et al. 2019). While these domains are empirically correlated, it remains unclear which are constitutive components of self-stigma, which are mediators or downstream consequences of it, and which are correlated outcomes manifesting through independent pathways.

This ambiguity creates a measurement paradox. When summative scales collapse cognition, affect, and behavior into a single index, individuals with sharply different regulatory profiles may receive identical total scores. For example, a person who strongly endorses self-devaluing beliefs but reports minimal shame may receive the same aggregate score as someone who experiences intense shame but rejects stigmatizing stereotypes. Yet these profiles plausibly imply very different participation trajectories and intervention needs.

The problem is not merely psychometric but conceptual. Aggregation obscures the possibility that different components of stigma operate through distinct regulatory pathways. Schwarzbald and colleagues (2021), cited

above, found that internalized stigma can be decomposed into latent components of ‘experiential stigma’ (including withdrawal and alienation) and stereotype endorsement, each with different predictive implications. Others have recently challenged the unidimensionality of self-stigma in domains as diverse as weight and bullying (Meadows and Higgs 2019; Song et al. 2025). Treating self-stigma processes as interchangeable under a single stigma score limits both theoretical precision and practical intervention targeting.

The present framework addresses this ambiguity by specifying self-stigma as a regulatory process with evaluative, prospective, and embodied pathways that may operate independently or in combination.

The paradox of adaptive self-exclusion

If self-stigma is the regulation of social participation in response to perceived exclusion, then it must be acknowledged that this regulation is often protective. Withdrawing from environments in which one is likely to be rejected, surveilled, or harmed is not irrational but adaptive. The person who avoids healthcare after repeated mistreatment, who declines to disclose a stigmatized status in a workplace known to be hostile, or who narrows their social world to those who already know and accept them is not exhibiting cognitive distortion. They are reading their environment and responding accordingly.

This observation is not incidental to the present theory – it is foundational. Self-stigma, as conceptualized here, does not presuppose error. Unlike frameworks that locate the harm of self-stigma in the endorsement of "incorrect" beliefs about the self, the relational-process model makes no claim about the accuracy of the appraisals involved. The individual who judges their participation to be unwelcome may be right. The expectation of rejection may reflect accumulated evidence. The felt incapacity to endure a demeaning interaction may be an honest accounting of one's physiological reserves. Self-stigma, in this view, is not a failure of reasoning but a mode of self-preservation within a social order that has made exclusion the predictable consequence of certain forms of difference.

Herein lies the central paradox: the process that protects the individual simultaneously accomplishes the broader social function of stigma. Self-exclusion shields one from immediate harm while fulfilling society's implicit expectation that stigmatized persons remove themselves from collective life. Protection and dispossession are not opposing outcomes. They are the same act, appraised from different vantage points. The individual experiences reprieve; the social order is reproduced.

This paradox has significant implications for how self-stigma is studied and addressed. It suggests that interventions targeting self-stigma cannot simply aim to correct beliefs or bolster self-regard. If self-exclusion is a rational adaptation to a hostile environment, then challenging the adaptation without altering the environment is not empowerment — it is exposure without protection. To tell someone they "deserve" to participate while leaving the conditions of participation unchanged is to invite harm. The relational-process framework thus demands that self-stigma intervention be understood as necessarily dual: attending both to the individual's regulatory orientation and to the social conditions that render self-exclusion adaptive.

It also suggests that the persistence of self-stigma in the face of belief change — a finding that has troubled existing models — is not paradoxical at all. If self-stigma is organized around participation rather than belief, then changing what one thinks about oneself does not automatically change what one permits oneself to do. The body remembers what the mind has revised. Expectancies, once sedimented through experience, do not dissolve upon reflection. The regulatory architecture of self-stigma is built for conservation, not correction — it is designed to keep one safe, and safety, once achieved through social withdrawal, is not easily relinquished.

Recognition of self-stigma's protective function does not diminish its harm. Rather, it sharpens the moral claim embedded in this analysis: that the conditions requiring such protection are themselves the primary site of injustice. Self-stigma is not pathology residing in the stigmatized individual. It is the trace of a social environment that has made full participation dangerous.

A call to revisit internalization

There is another possibility that the theory should suggest – that internalization and normative evaluation are not complementary processes but sequential steps in evaluative self-stigma. Current models suggest that stereotype endorsement, the precondition for internalization, is predictive of social isolation. It is imaginable that the reverse pathway is true: Those who find themselves isolated from society without evidence may need a retrospective explanation for their exclusion.

When an individual finds themselves unmoored from the "social we," (i.e., stigma cues gain salience), it is possible that the mind seeks to resolve the gap between their self-concept as a social being and their lived experience of isolation. Stereotype endorsement may thus function as a logical, albeit damaging, explanatory framework. Rather than the stereotype causing the exclusion, the exclusion necessitates the stereotype as a way to make the social world predictable again. The individual concludes: *"I am treated as a pariah; therefore, the stereotypes regarding my worthiness must be true."* In this "bottom-up" process, internalization is not the driver of the regulatory process but the cognitive fill used to rationalize a social environment that has already begun to constrict around the individual. Before there is "I am incompetent" as a belief about self, there is "I am out of place here" as a felt relational fact. The primary datum is the structure of the social field (exclusion, constricted roles, messages of non-belongingness, being treated as a burden), and only then does the mind internalize incompetence as a self-description. This would fit the pattern of "relationless relationality" proposed by Paskaleva-Yankova (2021): The affective atmosphere of alienation where one is physically present but intersubjectively excluded can create a profound state of cognitive dissonance. Stereotype endorsement may be one way of resolving this dissonance. That sociality, not internalization, might be the driver of self-stigma, as RPT posits, suggests that this reverse pathway requires further investigation.

Further areas of inquiry

This theory leaves some questions unanswered. It is not that they are unanswerable so much as they deserve deeper investigation than an introductory prospectus has room for. Stigma resistance is one such topic. Is it equivalent to attenuated self-stigma? Is it a distinct coping mechanism? RPT would likely categorize stigma resistance as the interruption of self-stigma processes: Stigma cues whose integration is actively challenged; evaluation that is reflectively disregarded; prospection that is overcome; embodiment that is rehabilitated. In short, the rejection of social exile. This supposes that self-stigma registers as unjust (cueing the “righteous anger” described in Corrigan and Watson’s (2002) self-stigma paradox and Thoits’ (2011) conceptualization of stigma resistance).

Second is the question of intervention (Sibley, Colston, and Go 2024). Despite its relational character, the theory offers more intrapersonal answers than structural ones. The primary implication is that self-stigma should be understood in terms of patterns rather than levels, and intervention should be tailored to profiles of evaluative, prospective, and embodied self-stigma rather than to the intensity of internalization. Consider a simplistic exemplification: One individual experiences high evaluative self-stigma but low prospective and embodied self-stigma. A second is especially attuned to the prospect of stigma but scores lower in evaluation and embodiment. A third feels stigma but has engaged in less evaluative and prospective reflection. The first person may well respond to second wave psychotherapeutic approaches (Cognitive-Behavioral Therapy) that challenge stereotype endorsement and self-devaluation cognitively. The others, however, retain a neutral or positive self-image; their rejection of stereotypical thinking need not be challenged. The person who engages in stigma prospection may benefit from defusing from rejection narratives, clarifying or reinforcing values, and committing to action (e.g., Acceptance and Commitment Therapy), or increasing tolerance for the felt distress of social engagement (e.g., Dialectical Behavioral Therapy). The third individual, whose stigma is autonomically embedded, likely requires a different approach. This person may perceive their non-participation as maladaptive but still feel frozen from action. What must be changed is more fundamental than belief: Dysregulation constitutes how stigma is felt, not just how it is noticed. Somatic experiencing may be appropriate in this scenario; safe and gradual experiential exposure is also worth considering. These examples

are provided philosophically rather than empirically. The constructs of RPT self-stigma must first be operationalized and measured before such inquiry evolves further.

Yet what is clear is that all three would benefit from structural intervention. We are reminded that self-stigma is not pathological but learned self-protective adaptation. The conditions that expose these individuals to the need for relational withdrawal must be addressed. It may be cynical to consider social stigma an insurmountable feature of society, but the human condition of typification, which allows for individuals to be grouped and devalued in the first place (Ainlay and Crosby 1986), will not evolve in our lifetimes. What can be changed, and what is fundamental to the question of stigma, is the restoration of equitable access to social inclusion. Civil rights legislation must account for the explicit and implicit barriers to values expression in the public arena. Local policies must support participatory openings and normalize inclusiveness. And stigma advocates must decenter challenging stereotypical beliefs, which are deeply entrenched in many, in favor of challenging the long taken-for-granted normative denial of belongingness.

Finally, empirical development of the theory is a necessary next step. Appendix A outlines measurement considerations consistent with RPT's ontology, and Appendix B specifies conditions under which the theory would be falsified.

Conclusions

Good social theory must meet two objectives: Synthesis and utility. It should honor its theoretical forebears while providing a new vantage from which to understand our social world. It is the intention of this proposal to build on, rather than detract from, the important work to date on self-stigma, while also providing an ontology that subverts what has felt commonplace in the field. It is hoped in the end that Relational-Process Theory might do some good for those who experience self-stigma; in that regard, it must ultimately be judged not on its theoretical insight but on its pragmatic merit.

Notes

1. Psychological constructs like shame, diminished self-esteem, and depression are the defined mediators between self-stigma and undesirable individual outcomes. See Hatzenbuehler (2009), Earnshaw and Chaudoir (2009), Corrigan, Rafacz, and Rüsch (2011), Yanos et al. (2008), and Molina et al. (2013) for examples. Relational-Process Theory does not discount that these constructs may manifest in self-stigma processes as mediators, moderators, or correlates, but they are not constitutive of self-stigma.
2. The hyphen in ‘self-stigma’ does important work. The English language has few *dvandva* compounds (in which the constituent words hold equal weight, e.g., owner-operator). Self-stigma can be considered a *dvandva*: It is not so much a form of stigma as the intersection of the social conditions of stigma with the self.
3. Perception, here, refers to the reception of sensory input with or without conscious reflection. This differs from ‘perceived stigma’ which is usually defined as stigma awareness arising from conscious reflection. Stigma cues can be processed and integrated pre-reflectively or somatically – cognition is not the sole route. The term *registered stigma* is used to highlight this difference.
4. Deservingness and belongingness both denote the legitimacy of inclusion or the normative permissibility of presence. They are used interchangeably. However, the connotations are slightly different. Deservingness is a hierarchical logic, signaling rank, worth, and status. This frame aligns with stigma traditions that emphasize power relations, e.g., Link and Phelan (2001) and Parker and Aggleton (2003). Belongingness is a boundary logic, signaling fit, comfort, and safety, aligning more closely with social identity theory (Tajfel and Turner 1979). This is a phenomenological distinction worth bearing in mind: the former is a sense of lowered social rank while the latter is a sense of being out of place.
5. A new term for what feels like a familiar concept is necessary. Internalized stigma has its own theoretical lineage and empirical grounding. Rather than conflate two concepts with related framings but intrinsic differences, RPT adopts terminology that acknowledges both internalized stigma and evaluative self-stigma as

distinct branches of stigma theory. This distinction not only respects the psychological foundation of internalized stigma but also centers the social-relational dynamics of evaluative self-stigma. The new terminology allows for clearer theoretical differentiation and more precise applications in both research and intervention strategies. The same can be said for anticipated stigma and prospective self-stigma.

Draft

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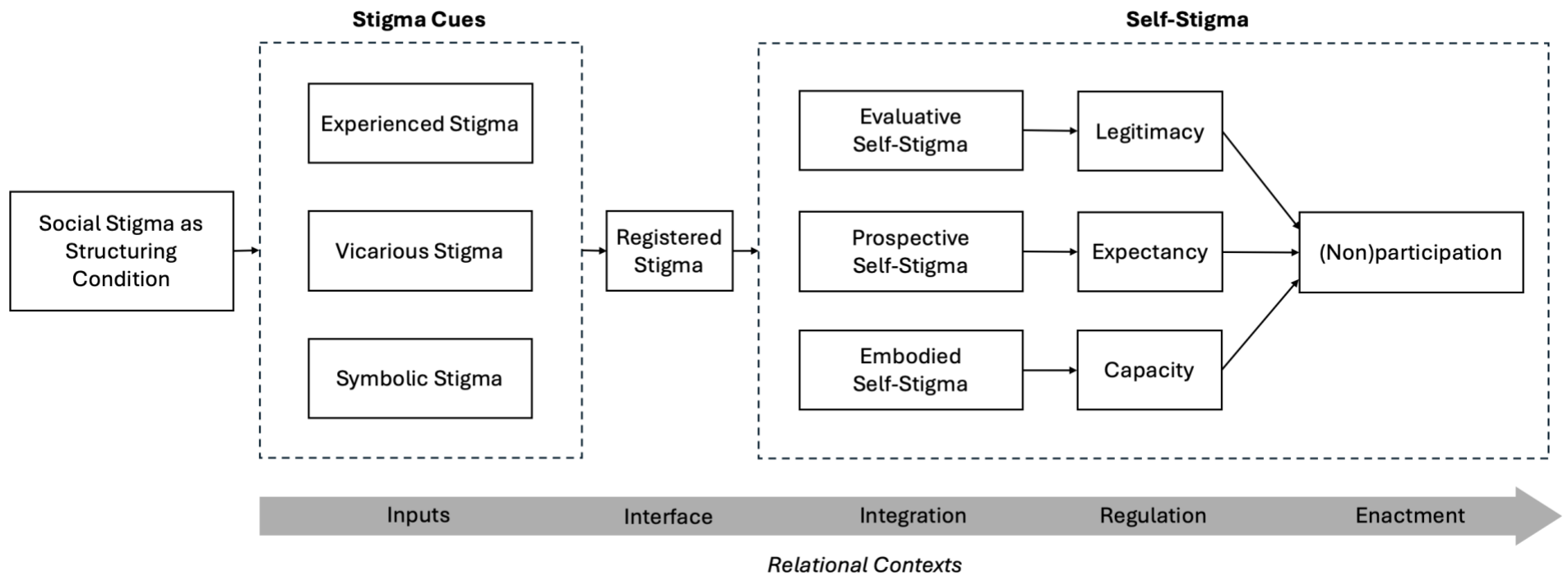


Figure 1. A conceptual model of the self-stigma process

APPENDIX A: Rethinking self-stigma measurement

Suitable markers of self-stigma are those that most proximally capture the process of self-exclusion and social non-participation. An appropriate scale item should affirmatively answer the question: *Does this item capture an individual's perceived legitimacy to participate, expectancy of acceptance, or felt capacity to engage in social life?* In other words, is the item explicitly social-relational?

- 1) The item should reflect the mark, not the identity.

Though social stigma is implicitly organized around group assignment, social identity salience and endorsement are not required to experience or engage in self-stigma. Measures should prioritize the behavior, health condition, phenotype, or other *mark* through which stigma is enacted rather than the identity suggested by the mark.

There is an additional pragmatic benefit: this approach circumvents the instability of identity language. Labels vary in salience, preference, and political charge. Some may reinforce stigma; others may feel artificial or externally imposed.

Consider two otherwise identical items:

“If someone found out I’m an addict, they’d be afraid to hang out with me.”

“If someone found out I’m a person who uses drugs, they’d be afraid to hang out with me.”

The first relies on a stigmatizing (though widely adopted) identity marker. The second uses a person-first construction that may not align with how respondents describe themselves. Both import identity politics into measurement.

An item anchored in the mark avoids this tension:

“If someone found out about my drug use, they’d be afraid to hang out with me.”

This formulation centers the social basis of stigma without requiring identity salience or endorsement.

- 2) The item should refer to social participation or belongingness.

Self-stigma, as defined here, is the regulation of social participation. Therefore, appropriate items must reference belonging, access, presence, opportunity, or legitimacy within collective life.

Items may be global or domain-specific so long as they anchor participation:

“I do not deserve the same opportunities as others.”

“When I consider participating in work activities, I expect to be excluded because of my substance use.”

“Participating in social gatherings feels too draining because of how I think I will be treated.”

Items that assess emotional distress, stereotype agreement, or global self-esteem without such social reference may correlate with self-stigma but do not constitute it.

- 3) The item should map onto a regulatory parameter.

Each question should be a distinguishable assessment of legitimacy, expectancy, or capacity, three connected yet distinct processes of integration and regulation. Items that blur regulatory processes likewise blur the respondent's unique patterns of non-participation, limiting intervention precision.

- 4) The item should not capture knowledge, perceptions, or experiences of stigma.

In treating self-stigma as a process, stigma cues undoubtedly shape one's regulatory patterns but do not constitute self-stigma. Experienced discrimination, perceived public stigma, and other self-stigma inputs may be of theoretical or applied interest but should be measured separately with existing, validated instruments. Symbolic stigma, especially – embedded in habitus and often internalized without conscious reflection – suggests that one's self-stigma profile need not present overt evidence of prejudice or discrimination. In other words, individuals might theoretically self-stigmatize without having a rational, reflective justification for doing so. Measuring other forms of stigma necessarily implies a different line of inquiry and/or intervention.

A simple diagnostic test evaluates a potential item's fit for inclusion: *If the broader social context were fully inclusive, would this item still make sense?* "I feel ashamed of myself" is an affective state that can exist in an inclusive world. "I do not deserve to be in professional settings" only makes sense in relation to norms of exclusion. The latter may reflect self-stigma. The former, by definition, does not.

APPENDIX B: Refutation conditions

To ensure scientific rigor, RPT must be amenable to empirical refutation. The theory posits that self-stigma operates as a dynamic regulatory response to socially structured environments rather than solely as an internal cognitive-affective state. Accordingly, meaningful incremental variance in social participation should be observed beyond the effects of internalized stigma as currently operationalized. The theory would be challenged under the conditions outlined in Table 1.

Table 1. Empirical conditions that would challenge the Relational-Process Theory

Prediction	Consistent finding	Inconsistent finding
Regulatory architecture	Prospective (expectancy-based) and embodied (capacity-based) regulatory processes explain significant incremental variance in social participation beyond evaluative processes (e.g., internalized beliefs, shame, self-esteem).	Prospective and embodied measures fail to explain additional variance in participation once evaluative cognitive-affective processes are adequately modeled.
Evaluation without endorsement	Normative evaluations of non-belongingness or undeservingness are observed among stigmatized individuals who do not endorse stereotypes as self-descriptive.	Legitimacy appraisals are observed only as a downstream consequence of stereotype endorsement and self-application, with little or no systematic evidence of evaluative withdrawal absent stereotype concurrence.
Constraint without cognition	Indicators of embodied burden (e.g., chronic vigilance, physiological strain, regulatory depletion) are associated with participatory withdrawal even after accounting for evaluative self-stigma and anticipated rejection.	Associations between embodied burden and participation are fully explained by cognitive-affective processes (e.g., internalized beliefs or anticipated stigma), with no independent contribution of embodied pathways.
Participatory openings	Expansion of participatory openings (e.g., low-barrier hiring, non-judgmental service environments, inclusive social spaces) is associated with increased engagement over time, even when global self-evaluations (e.g., self-esteem) remain unchanged.	Participation remains unchanged following sustained creation of inclusive, low-barrier opportunities once baseline cognitive-affective variables are accounted for.

Domain-specific values expression	Individuals who appear disengaged in stigma-exposed domains (e.g., formal employment) continue to enact the same values in protected or low-stigma contexts; within-person participation varies systematically by stigma exposure level.	Self-stigma is associated with uniform, cross-context diminishment of values expression, with little evidence of domain-specific variation linked to stigma exposure.
Persistence after label removal	Participatory constraint persists following formal removal or attenuation of the stigmatized label (e.g., remission, recovery), and residual disengagement is not fully explained by normalization of self-beliefs.	Changes in self-beliefs fully account for changes in participation following label removal, with minimal residual constraint once evaluative processes normalize.